A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

18 August 2026

To Whom It May Concern

Dear To Whom It May Concern,

Re: Ms Mackenzie Woeste
DOB: 24/08/1992 Phone: 0416519307
Address: 3/8 Newcastle Street, PRESTON, VIC, 3072
Email: Mackenzie Woeste

I have met Mackenzie on one occasion to perform an open access Gastroscopy. Histologically there was evidence of EoE. At the time when I conducted a post procedure phone call to update her on the histology results she denied symptoms of EoE. This of course may not be the case now as this procedure was performed some time ago. Histology, report and prior letters attached.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

WOESTE, MACKENZIE CLAIRE : SVP 864676

DOB: 24/08/1992 Age: 33 years

Unit 3 / 8 Newcastle St, Preston 3072

Results from 06/04/2024 ..... Episode# 24P280966 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2024B008445 **CLINICAL NOTES** Duo N, Gas N, oesophagus N. Iron deficiency. Random colon N. Anal polyp = ? Any histo. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophagus". Three tan mucosal fragments 1-3mm. A1. 2. Specimen labelled "Gastric". One tan mucosal fragment 6mm. A1. 3. Specimen labelled "Duodenum". Five tan mucosal fragments 3-6mm. A1. 4. Specimen labelled "Random colon". Three tan mucosal fragments 2-5mm. A1. 5. Specimen labelled "Incus bx ??". One tan mucosal fragment <1mm. A1. Specimen may not survive processing. **MICROSCOPIC DESCRIPTION** 1. Sections show fragments of stratified squamous oesophageal mucosa. There is suprapapillary plate thinning, elongation of papillae and basal cell hyperplasia. Eosinophils exocytosis is noted associated with spongiosis. There is up to 120 eosinophils in one high power fields. No dysplasia is noted. 2. Sections show fragments of gastric mucosa including muscularis mucosa. There is mild chronic inflammation and regenerative changes. No acute inflammation, ulceration or granuloma formation is seen. No intestinal metaplasia or dysplasia is

All appointments and enquires

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Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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present. Helicobacter pylori-like microorganisms are not identified on the H&E sections. 3. Sections show fragments of duodenal mucosa including muscularis mucosa and Brunner's gland. The villous architecture is preserved. No intra-epithelial lymphocytosis is seen. No acute inflammation, ulceration or granuloma formation is noted. No dysplasia is present. 4. Sections show fragments of large intestinal mucosa including muscularis mucosa. The crypt architecture is preserved. No acute inflammation, ulceration or granuloma formation is seen. No thickening of the subepithelial collagen plate is noted. No intra-epithelial lymphocytosis is present. No dysplasia is identified. 5. Histology demonstrates a minute superficial fragment of stratified squamous mucosa only. The biopsy is insufficient for proper histological assessment. Where accessible, no full thickness atypia is seen. **PATHOLOGIST'S OPINION 1. Oesophageal biopsy: Eosinophilic oesophagitis (120 eosinophils/1HPF, benign. 2. Gastric biopsy: Mild chronic chemical gastropathy, benign. 3. Duodenal biopsy: Within normal morphological limits. 4. Random colonic biopsy: Within normal morphological limits. 5. Incus bx: Minute superficial fragment of squamous mucosa only, insufficient for proper histological assessment.** Pathologist: Dr Zakariya Kanaan

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